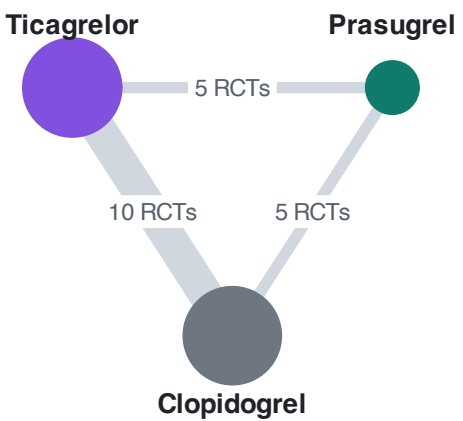
Revisiting P2Y12 inhibitors after acute coronary syndromes: a Bayesian network meta-analysis

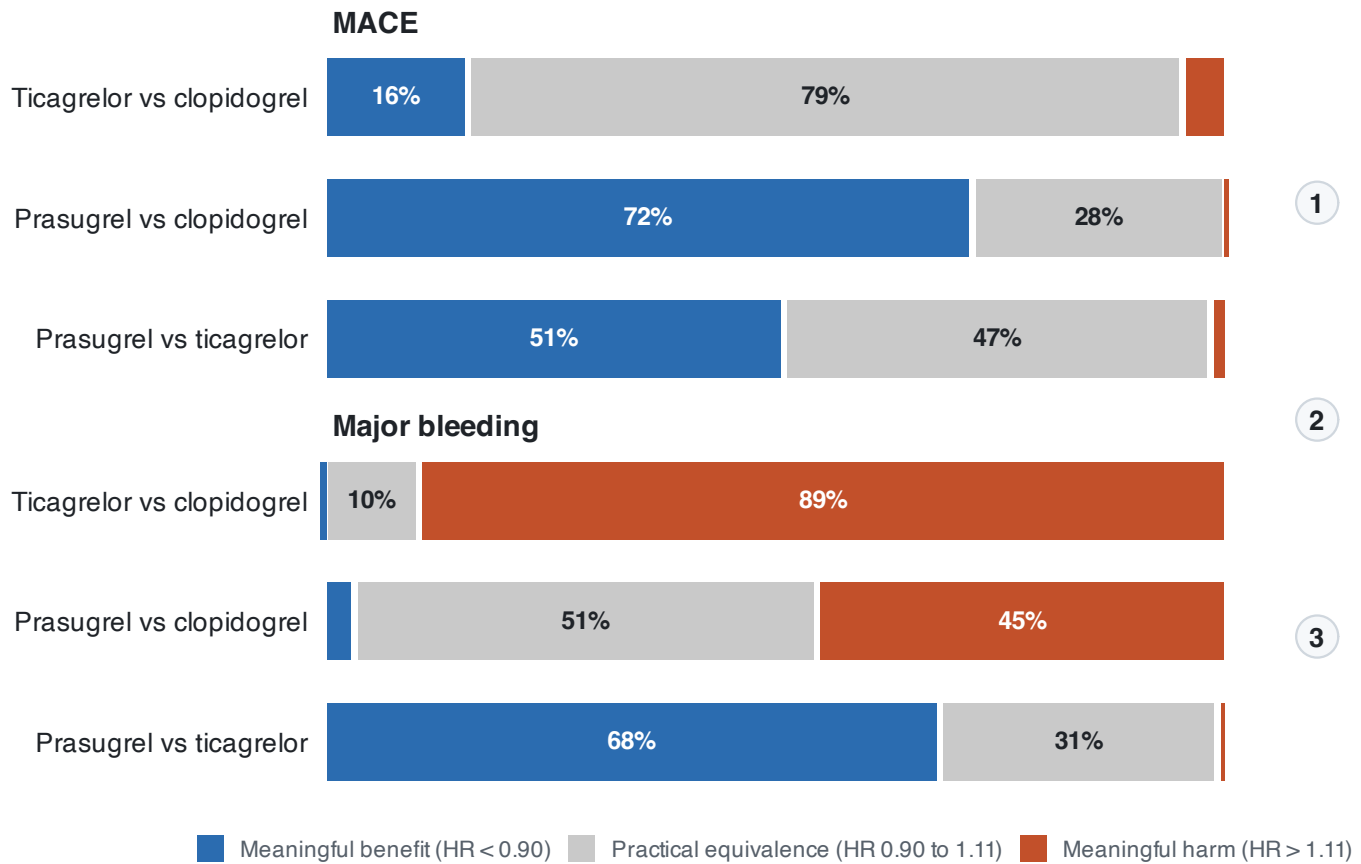
The evidence

20 randomized trials, 77,714 patients
ACS, all regimens with aspirin



What the Bayesian analysis shows

Posterior probability that the first-named drug is meaningfully better, equivalent or meaningfully worse



Clinical message

1. No definitive hierarchy: the probability that prasugrel is meaningfully better than ticagrelor for MACE is 51%, roughly a coin toss
2. Ticagrelor and clopidogrel are probably equivalent for MACE (79%), but ticagrelor probably causes more bleeding (89%)
3. Clopidogrel remains a reasonable choice, particularly for older and high-bleeding-risk patients

MACE, death, myocardial infarction or stroke. HR, hazard ratio. Random-effects network meta-analysis with a complementary log-log link and follow-up offset.